New Somerset Hospital
Department of Anaesthesia

PATIENT STICKER
Name .....
Folder no. .....
Date of birth .....

REGIONAL ANAESTHESIA CONSENT — Lower Limb Nerve Block

1. PLANNED PROCEDURE & BLOCK

Procedure ..... Side: Left Right Bilateral

Block(s): ☐ Fascia iliaca ☐ PENG ☐ Femoral ☐ Lat. femoral cut. ☐ Adductor canal
☐ IPACK ☐ Obturator ☐ Lumbar plexus ☐ Sciatic (proximal) ☐ Popliteal sciatic
☐ Ankle Other .....

Planned with: ☐ Block alone ☐ Sedation ☐ GA ☐ Spinal ☐ Site marked

Patient has capacity to consent: Yes No If no, consent given by (name / relationship) .....

2. BENEFITS DISCUSSED

why a block is recommended for this patient

- ☐ Excellent pain relief at the operation site (often 8–24 h)
- ☐ Less opioid needed: less nausea, vomiting, drowsiness, itch, constipation
- ☐ Hip fracture: analgesia before surgery, easier positioning for spinal, may reduce delirium
- ☐ Motor-sparing blocks preserve strength → earlier walking & physio
- ☐ Avoid GA (e.g. foot/ankle surgery with popliteal / ankle block)
- ☐ Earlier eating, drinking, mobilising and discharge

Other benefit .....

3. RISKS DISCUSSED

most patients have no complications · details and figures in the patient leaflet

General — all nerve and plane blocks		
☐	**Common** (1 in 10–100)	Prolonged numbness / weakness (expected 8–24 h) · Partial or failed block · Rebound pain as the block wears off · Bruising / tenderness · Temporary tingling or numbness
☐	**Uncommon** (1 in 100–1 000)	Bleeding / haematoma
☐	**Rare** (1 in 1 000–10 000)	Local anaesthetic toxicity (fits, heart rhythm) · Permanent nerve damage (> 6 months)
☐	**Very rare** (< 1 in 10 000)	Infection · Severe allergic reaction
Block-specific		
☐	**Common** (1 in 10–100)	Thigh weakness → falls
☐	**Uncommon** (1 in 100–1 000)	Foot drop / heel pressure injury
☐	**Rare** (1 in 1 000–10 000)	Masked compartment syndrome · Lumbar plexus: epidural spread, deep haematoma
☐	Other risk discussed	.....

4. ALTERNATIVES DISCUSSED

- ☐ Oral / IV analgesia (incl. opioids or PCA)
- ☐ Local anaesthetic infiltration by surgeon
- ☐ Spinal / epidural
- ☐ General anaesthetic without a block
- ☐ A different block

5. AFTERCARE ADVISED

- ☐ Do not stand or walk unassisted until strength returns
- ☐ Protect the numb area from heat, cold, pressure and injury
- ☐ Regular analgesia before the block wears off
- ☐ Report pain out of proportion / tight swollen limb
- ☐ Report numbness / weakness lasting > 48 h

6. CONFIRMATION

Right to refuse explained (no effect on surgery or care): Yes No ☐ Questions answered

Patient leaflet: QR shown Printed copy Declined

Interpreter: Not needed Used Language ..... Name .....

Patient gives consent for the block(s) above: YES NO

Patient leaflet

Doctor ..... MP no. ..... Signature .....

Date ..... Time .....

Patient leaflet: omicronblade.github.io/Anaes-Doc-Repo/anaesthesia-forms/leaflets/regional-lower-limb.html

SASA/SASRA Guidelines 2016 · McCombe & Bogod, Anaesthesia 2021 · frequencies approximate